

Example Peer Recovery Services, Inc.

Surveyor Interview Bank

What the surveyor asks, what a strong answer sounds like, and what sinks you

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Surveyor Interview Bank

Written from the surveyor's chair. Every question below is one a CARF surveyor realistically asks, grouped by who gets asked. For each: **what they are really testing, the shape of a strong answer, what sinks you, **and** the document they may ask to see next**.

How to use this — read this part twice

These are not scripts. A surveyor has interviewed hundreds of agencies and can tell a memorised answer from a real one in about four seconds. A rehearsed line delivered by someone who does not do the thing is worse than an honest "I'd have to check" — it turns a documentation gap into a credibility problem, and credibility problems spread to every other answer.

Use the model answers as the **shape** of a good answer: what to include, in what order, at what level of detail. Everything in [SQUARE BRACKETS] is a real fact the person has to supply from their own work. If a staff member cannot fill the brackets, that is not an interview problem — it is the finding, and you have found it before the surveyor did. Fix the practice, then the answer takes care of itself.

Three rules to give every person before survey week:

1. **Tell the truth.** Including "we don't do that well yet, and here's what we're doing about it."
2. **"I don't know, but I know where to find it"** is a good answer. Guessing is not.
3. **Never say what you think they want to hear.** Say what actually happens.

And one absolute rule: never coach a person served. Telling someone what to say to a surveyor is the fastest way to lose an accreditation, and it is a rights violation on its own terms. Ask people whether they are willing to talk. Tell them it is voluntary, that they can stop, and that nothing changes either way. Then leave them alone.

A. The Chief Executive / Executive Director

A1. Walk me through your organizational chart. Who is accountable for quality?

Testing: whether leadership is real and assigned, or a box on paper (1.A).

Strong answer: "[Name] is the QI Coordinator; that's a named role in their job description, not something they picked up. They report to me. We meet as a leadership team [monthly] and quality is a standing item — here are the last twelve months of minutes. In a shop our size one person wears several hats, so [name] is also [role], and that assignment is written into the job description rather than assumed."

Sinks you: "We all handle quality." Nobody is accountable, so nobody is.

They may ask for: org chart, signed leadership job descriptions, leadership minutes.

A2. What does your strategic plan say, and what data went into it?

Testing: whether the plan was built from evidence or from a weekend off-site (1.C).

Strong answer: "Three goals for [period]. Before we wrote it we did an environmental scan — here it is. It pulled in the satisfaction analysis, county demographics for [counties], our financial projection, workforce supply for certified peers, the risk register, and last year's performance analysis. Goal 2 exists specifically because

[finding from the scan]."

Sinks you: a plan with no documented inputs, or one nobody has reviewed since it was written.

They may ask for: strategic plan, environmental scan worksheet, board minutes approving it.

A3. When did your governing body last review organizational performance?

Testing: governance is exercised, not nominal (1.B).

Strong answer: "[Date]. It's in the minutes — they reviewed the quarterly dashboard, the incident summary and the financials. They asked about [specific thing], and we came back to them in [month] with [answer]."

Sinks you: minutes that record attendance and nothing else.

They may ask for: twelve months of board minutes.

A4. What are your top three risks and what are you doing about each?

Testing: risk management is live, not a binder (1.G).

Strong answer: "Staff safety on home visits, payer concentration, and turnover of certified peer staff. For the first: [control]. For the second: [percentage] of our revenue is [payer], so [mitigation]. Third: [action]. They're on the risk register with owners and dates, and we reviewed it [date]."

Sinks you: naming generic risks ("HIPAA", "funding") with no owner and no date.

They may ask for: risk management plan, risk register, insurance declarations.

A5. Show me your budget and the assumptions behind it. Who reconciles the bank statement?

Testing: financial planning and separation of duties (1.F).

Strong answer: "Budget approved [date] by [governing body]. The assumptions are written on the front page — units per month, our average rate, our denial rate, and collection lag, all from actual history. [Name] writes cheques; [different name] reconciles the account and I review the reconciliation. In an agency this size we can't fully separate every duty, so the compensating control is [describe] and it's written into the financial policy."

Sinks you: one person doing everything with no compensating control documented.

They may ask for: budget with assumptions, twelve months budget-to-actual, signed reconciliations.

A6. What did last year's performance analysis tell you, and what changed because of it?

Testing: the loop closes (1.M). This is the single most important question in the survey.

Strong answer: "Three things. [Measure] missed target at [number] against [target]. We ran an improvement project — the charter is here, baseline [x], end result [y] — and we adopted the change permanently by [what changed]. [Measure] beat target so we raised it. And [measure] surprised us: [what and why]."

Sinks you: an analysis that lists numbers with no conclusions, or improvement "actions" with no before-and-after data.

They may ask for: the annual analysis, PI project charters, minutes.

A7. How did the people you serve find out about those results?

Testing: distribution — the most-missed requirement in the manual (1.M).

Strong answer: "A one-page plain-language summary, [date]. It's posted at [location], we handed it out, and we went through it at the [month] group. Here's the distribution log and here's the summary."

Sinks you: "It's available if they ask." That is not distribution.

They may ask for: the summary, the distribution log, a person served who remembers it.

A8. How do you know your staff are competent, and not just trained?

Testing: the training/competency distinction (1.I). Very commonly failed.

Strong answer: "Training is a sign-in sheet. Competency is a separate assessment — [supervisor] observes an actual service contact, reviews the person's documentation, and rates each competency met or not yet met. Anything 'not yet' gets a development action and a reassessment date, and they don't carry that responsibility alone until it's met. Here are three completed assessments including the observation records."

Sinks you: producing training rosters when asked about competency.

They may ask for: competency checklists, direct observation records.

A9. What's your biggest weakness right now?

Testing: self-awareness. There is no right answer except an honest one.

Strong answer: name a real one, then the plan. "[Real weakness]. We found it in [record review / incident analysis / staff feedback], and the action is [x] by [date]."

Sinks you: "I can't think of one." Nobody believes it, and it makes every other answer suspect.

A10. Tell me about a complaint you received and what happened.

Testing: the grievance process is used and not feared (1.J).

Strong answer: walk one through end to end with dates: received, acknowledged in writing within three business days, investigated by someone not involved, written decision within thirty days, and what changed systemically.

Sinks you: "We've never had one." Almost never believed, and it usually means informal concerns aren't being logged.

They may ask for: grievance log, one complete grievance file.

A11. How many people do you serve, and how did you decide your caseload cap?

Testing: staffing is reasoned, not accidental (2.A).

Strong answer: "[N] active. The cap is [number] per specialist. We set it from travel time across [counties], acuity mix, documentation time, and what our incident and outcome data looked like at higher numbers. It's in the staffing plan and we review it annually against turnover and satisfaction."

Sinks you: a number with no reasoning behind it.

B. The Governing Body

B1. What are your duties as a board member?

Strong answer: mission, strategic plan, budget approval, hiring and annually evaluating the CEO, major policy, reviewing performance and risk, legal and ethical operation. "They're written down — here they are."

Sinks you: "We advise." Advising is not governing.

B2. When did you last evaluate the CEO, and against what?

Strong answer: "[Date], against the expectations we set the year before. Here's the written evaluation. We set [n] expectations for the coming year."

Sinks you: an informal chat with nothing in writing.

B3. How does the board assess its own performance?

Strong answer: "Annual self-assessment, [date]. It showed [finding] and we [action]."

B4. What do you know about the quality of services?

Testing: whether the board sees quality data at all.

Strong answer: cite an actual number from the last dashboard and what the board asked about it.

Sinks you: a board that only sees finances.

B5. How are conflicts of interest handled here?

Strong answer: "Everyone signs a disclosure annually. When [example] came up, [member] declared it, left the discussion and abstained — it's recorded in the minutes."

C. The Program Director

C1. Describe the program. Who is it for and what do you actually do?

Strong answer: the written program description in your own words, then a concrete example of a person's week. Name the population, settings, hours, counties, staffing.

Sinks you: a description that doesn't match what the surveyor sees on site.

C2. How long from referral to first contact? How do you know?

Strong answer: "Our standard is [N] business days. Last quarter our mean was [number] — it's on the referral log and it's measure A1 on the dashboard. [If it slipped:] It went to [number] in [quarter] because [reason], and we [action]."

Sinks you: a guess. This is a measured number or it is nothing.

C3. What happens to someone you can't serve?

Testing: the referral-out obligation, routinely missed (2.B).

Strong answer: "We tell them why in plain language, give at least two specific alternatives with contact details, and offer to make the connection. It's all on the referral-out log — here's last quarter."

Sinks you: "We tell them we can't help." That is a finding.

C4. Show me how a person-centered plan gets made. Who writes the goals?

Strong answer: "The person does, in their own words — we write them in quotation marks. We start from their strengths, needs, abilities and preferences, then build measurable objectives around what they said. They sign it and get a copy."

Sinks you: goals in program language ("will comply with treatment").

C5. What happens after a critical incident?

Strong answer: the sequence with timeframes — immediate report to supervisor, written report by end of next business day, external reports within their required windows, supervisor review within five business days, debrief with the person within two business days and with staff within five, plan updated if needed, quarterly aggregate analysis.

Sinks you: incident reports filed and never analysed.

C6. What happens when someone just stops showing up?

Strong answer: "At least three attempts by two different methods over fourteen days, including a letter, all logged. Then a discharge summary — including for unplanned exits — and a follow-up attempt at [N] days. We review unplanned exits quarterly for patterns; last quarter's showed [finding]."

Sinks you: records closed silently.

C7. How do you handle it when a peer specialist knows a participant from their own recovery community?

Testing: the peer-specific question that separates a real peer program from a relabelled one (3.PEER).

Strong answer: "It happens, and we expect it to. They disclose it in writing before or immediately on assignment. The supervisor writes a management plan — reassign, or a written boundary agreement about what happens when both are in the same meeting, or increased supervision. We never pretend it isn't there, and we never restrict a staff member's own recovery participation."

Sinks you: "That doesn't happen here." In a small recovery community it always does.

C8. Do your staff document services they didn't deliver?

Testing: documentation integrity. Answer directly.

Strong answer: "No, and we check. Quarterly record review compares documented service against what was billed — date, time, duration, location, rendering provider. Anything that doesn't match goes to the Compliance Officer within a business day and gets repaid."

D. The Supervisor

D1. How often do you supervise, and what's documented?

Strong answer: "Twice monthly for the first ninety days, monthly after. Every session is documented on the supervision note — cases, self-disclosure, boundaries, safety, documentation, and the staff member's own wellness. Both sign."

Sinks you: supervision that happens but isn't written down. It didn't happen.

D2. Have you ever watched someone deliver a service?

Testing: direct observation, the weakest link in most competency systems (1.I).

Strong answer: "Yes — within ninety days of hire and at least annually. Here are the observation records with the date, setting, what I saw and the rating."

Sinks you: "I review their notes." That's document review, not observation.

D3. How do you supervise self-disclosure?

Strong answer: "Every peer specialist has a written disclosure plan — what they'll share, what they won't, and how they'll answer something outside it. It's a standing item in every supervision: what did you share, whose need did it meet, would you do it again. New staff get one disclosure directly observed in their first ninety days."

Sinks you: "We tell them to use judgement."

D4. What do you do when a staff member is struggling with their own recovery?

Testing: peer workforce wellness (3.PEER), and whether the agency punishes honesty.

Strong answer: "Their wellness plan says what they want me to do if warning signs show — they wrote it. I offer support first: supervision, schedule flexibility, caseload adjustment, EAP or peer consultation. It's kept in the confidential file, not the personnel file. We follow employment law and any credentialing obligation, and we don't terminate someone for having the condition this job asks them to have lived experience of."

Sinks you: an answer that starts with discipline.

D5. What did your last record review find, and what did you do?

Strong answer: a real finding, the correction, and the verification date. "And the same finding for the same person twice becomes a competency conversation, not another memo."

E. Peer Support Specialists — the most important interview in the survey

Surveyors talk to direct staff because leadership can describe a system that direct staff have never experienced. If these answers and the CEO's answers don't match, the surveyor believes these.

E1. What is peer support? What is it not?

Strong answer: "It's mutual. I've been where they are — I have my own lived experience of [general terms are fine]. I don't assess, diagnose, treat, or tell people what to do. I'm not their therapist and I'm not their case manager. I walk alongside them while they work on what *they* want."

Sinks you: describing clinical work, or being unable to say what the job is not.

E2. What makes you qualified for this job?

Strong answer: "My lived experience of recovery, plus my [credential], plus the training and competency assessment here."

Note: nobody has to disclose personal detail to a surveyor. "Lived experience of mental health and substance use recovery, and I'm certified" is complete.

E3. Give me an example of when you shared your own story. Why did you share it?

Strong answer: a real example with a purpose. "She was scared to appeal a benefits denial. I told her I'd had one overturned and what it felt like to walk in. I kept it short — the point was that it's survivable, not my story. She filed the appeal."

Sinks you: an example where the peer specialist was the centre of it.

E4. What do you do if someone tells you they stopped their medication?

Testing: scope limits (2.E). Every peer program is asked this.

Strong answer: "I don't advise them either way — that's between them and their prescriber. I'd ask what's going on, offer to help them call the prescriber that same day, help them write down what they want to say, and go with them if they want. I'd document what they told me and what I did, and tell my supervisor."

Sinks you: "I'd tell them to keep taking it." Well-meant, and outside scope.

E5. You're at a home visit and you feel unsafe. Exactly what do you do?

Strong answer: "I leave. I don't need permission and I won't be disciplined for it. I call my supervisor the same day, we document it as an incident, and we redo the visit safety plan — maybe two-person, maybe a public place, maybe telehealth."

Sinks you: any answer that involves staying to avoid trouble.

E6. Someone escalates in your car. What do you do?

Strong answer: "Pull over safely. I don't try to restrain anyone — we don't do that at all. Give space, stay calm, offer choices, use what's in their crisis plan. If there's a risk of serious harm I call 911 or mobile crisis. Then incident report and a debrief."

E7. You see a participant at your own recovery meeting. What now?

Strong answer: "That's a dual relationship and I disclose it in writing. My supervisor and I write a plan. Usually we agree in advance that neither of us brings up the service relationship in the meeting — I'm there as a member, not as staff. My own recovery doesn't stop because of my job."

E8. A participant sends you a friend request. What do you do?

Strong answer: "Decline it, and mention it in supervision. I don't connect with people I serve on personal accounts, I never post anything that could identify anyone, and service communication goes through the work phone or the record system."

E9. Where are the emergency procedures? What do you do in a fire here?

Strong answer: point at them, then describe the route and the meeting point.

Sinks you: having to look for them.

E10. Who do you report suspected abuse to, and how fast?

Strong answer: "My supervisor immediately, and the report to [agency] goes in within [timeframe]. I'm a mandated reporter — I report even if I'm not sure, and it doesn't wait for permission."

E11. How do you know what a participant's goal is?

Strong answer: "It's on their plan in their own words — we wrote it together. Every note I write links to one of those goals. If what we did doesn't fit a goal, that's a sign the plan needs updating, and I raise it."

E12. How do you take care of yourself in this job? Does the agency help?

Testing: whether wellness support is real (3.PEER).

Strong answer: a specific example. "I have a wellness plan I wrote with my supervisor. [Concrete support the agency actually gave.]"

Sinks you: "They say self-care is important." That's a poster, not a support.

E13. If you had a concern about the agency, who would you tell? What if it was about your boss?

Strong answer: names a route past their own supervisor — the Compliance Officer, the CEO, the board, or anonymously — and knows it's posted. "And nobody gets punished for it."

E14. What's your caseload? Is it manageable?

Answer honestly. If it isn't, say so and say what you've raised. A surveyor would rather hear a real number and a real conversation than a brave face.

E15. Have you ever been asked to document something that didn't happen?

Strong answer: "No." If yes — say so. Concealing it is far worse than the finding.

F. Persons Served

Never coach these answers. Ask people if they're willing to talk, tell them it's voluntary, and leave it there. What follows is what surveyors ask so you know what your practice has to produce — not what to tell anyone to say.

#	The surveyor asks	What it tests	What your practice has to produce
F1	How did you get connected here?	Access (2.B)	A referral route people can describe
F2	How long did you wait?	Access timeframes	A wait people don't experience as long
F3	Did someone explain your rights? Do you have a copy?	1.J	Orientation that actually happened, with a copy handed over
F4	If you had a complaint, what would you do?	1.J — the one they always ask	Every person able to name a route
F5	Would anything bad happen if you complained?	Non-retaliation	A confident "no"

#	The surveyor asks	What it tests	What your practice has to produce
F6	Whose goals are on your plan? Who picked them?	2.C	Goals the person recognises as theirs
F7	Can you show me your plan? Are those your words?	2.C	Plans in the person's voice, and they have a copy
F8	Can you say no to something here?	3.C self-determination	People who know they can
F9	Has anyone pressured you about medication or how you do your recovery?	Pathway neutrality	A clear no
F10	Where do you meet your peer specialist?	Community integration	Real community settings
F11	What's changed in your life since you started?	Effectiveness	Something concrete the person names
F12	Is there anything you needed that they couldn't help with?	Honest gaps	An answer you can live with
F13	Do you know what happens when services end?	2.D	Transition planning from day one
F14	Do you know how to reach someone in a crisis?	2.F	A crisis plan the person actually holds

If a person served says something unflattering, that is not a disaster. Surveyors expect real people to say real things. What they watch is whether the agency's own data already knew about it. "Yes — that came up in our satisfaction survey too, and here's what we did" is a strong moment. Being blindsided is the weak one.

G. Administrative and Billing Staff

G1. Walk me through what happens between a service being delivered and a claim going out.

Strong answer: the sequence, with the check in it: note completed within [N] hours by the person who delivered the service, pre-billing review against date/time/duration/location/rendering provider, then submission.

G2. What happens if you find a claim that shouldn't have been paid?

Strong answer: "It goes to the Compliance Officer, we void or repay within the payer's timeframe, and it gets logged. We had [n] last year and here's the log."

G3. Who can see the records of persons served?

Strong answer: role-based, minimum necessary, unique logins, quarterly access review.

G4. What do you do if someone calls asking whether a person is enrolled here?

Strong answer: "I don't confirm or deny. I take a number and route it to the Privacy Officer. Confirming enrollment is a disclosure."

H. The five questions that expose a gap fastest

Run these first in your mock survey. If any one of them stumbles, you are not ready.

1. **"Show me the data."** Policies are easy. Four quarters of real numbers is not.
2. **"What did you do about it?"** Finding a problem is half. Closing it and verifying it is the standard.
3. **"How did the people you serve find out?"** Distribution is the most-missed requirement in the manual.
4. **"Show me the drill on the evening shift."** Drills almost always cover one shift.
5. **"Read me a goal in the person's own words."** Goals in staff language are the most common 2.C finding.

I. Running the mock survey

- Do it **at least sixty days out**, so there is time to fix what it finds.
- Have someone who does **not** work in the program ask the questions.
- **Write down the actual answers**, not whether they "did fine."
- Score each: **Confident and correct** / **Correct but hesitant** / **Wrong or blank**.
- Anything not "confident and correct" is a training action with an owner and a date, tracked on the Corrective Action Tracker like any other finding.

- Re-test the ones that failed, closer to the survey.

Person	Role	Date interviewed	Questions asked	Confident	Hesitant	Wrong/blank	Actions
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Mock survey conducted by: _____ Date: _____

Findings transferred to the Corrective Action Tracker on: _____

Re-test completed: _____